

Clinical Study Summary Report (CSSR)

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1. Administrative & Study Identification

Full Study Title: Real-Life Effectiveness and Safety in Participants With Advanced/Metastatic Renal Cell Carcinoma Starting 1st Line Nivolumab and Ipilimumab Combination Therapy or Nivolumab Monotherapy After Prior Therapy

Short Title/Acronym: NORA Trial (CA209-653)

Protocol Number: CA209-653

NCT Number: NCT02940639

Sponsor Name: Bristol Myers Squibb

Investigational Product: Nivolumab monotherapy / Nivolumab and Ipilimumab combination therapy

Phase of Development: Phase IV / Observational (Non-interventional)

Medical Monitor: Bristol Myers Squibb Clinical Operations

2. Study Synopsis & Rationale

2.1 Study Objective * **Primary Objective:** To estimate overall survival (OS) over a 5-year follow-up period among adult participants with advanced/metastatic kidney cancer, starting 1st line nivolumab and ipilimumab combination therapy or nivolumab monotherapy after prior therapy, in real-life conditions in Germany. * **Secondary Objectives:** To evaluate Progression-Free Survival (PFS), Objective Response Rate (ORR), safety profile, and health-related quality of life (HRQoL) using EQ-5D scores in a real-world setting.

2.2 Background & Rationale To evaluate the real-life effectiveness, long-term survival, and safety of nivolumab alone or in combination with ipilimumab in patients with advanced or metastatic Renal Cell Carcinoma (aRCC) treated according to standard clinical practice in Germany. This prospective registry addresses the gap between highly controlled interventional clinical trials and real-world outcomes.

3. Study Design & Methodology

3.1 Design Framework * **Study Type:** Observational (Prospective, non-interventional, multicenter) * **Control Type:** Single-arm (Real-life cohort assignment) * **Blinding:** Open-label (None) * **Randomization:** N/A (Treatment is based on routine physician clinical decision)

3.2 Planned Enrollment & Duration * **Target N\$:** 323 participants. * **Number of Sites:** Multicenter (Germany). * **Estimated Study Start:** Late 2016. * **Estimated Primary Completion:** October 2024 (Study Completed).

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Adult participants ≥ 18 years of age at the time of treatment decision. 2. Histologically or cytologically confirmed diagnosis of advanced/metastatic Renal Cell Carcinoma (RCC). 3. Physician has already decided to initiate a treatment with 1st line nivolumab and ipilimumab combination therapy in intermediate/poor risk participants according to IMDC score, OR with nivolumab monotherapy after prior therapy for the first time, according to the label approved in Germany.

4.2 Exclusion Criteria 1. Diagnosis of a cancer other than advanced/metastatic RCC within the past five years that requires systemic or other treatment. (Patients curatively treated >5 years ago without recurrence or prostate cancer patients in active surveillance are permitted). 2. Participants previously treated with nivolumab and/or ipilimumab. 3. Participants currently included in an interventional clinical trial for their locally advanced or metastatic RCC.

5. Intervention & Treatment Plan

5.1 Investigational Regimen * **Dosage/Frequency:** Nivolumab monotherapy or Nivolumab + Ipilimumab combination therapy administered per routine clinical practice and the approved label in Germany. * **Route of Administration:** Intravenous (IV). * **Duration of Treatment:** Continued until disease progression, unacceptable toxicity, or routine real-world discontinuation parameters are met, followed by a 5-year OS follow-up.

5.2 Concomitant & Prohibited Therapy * **Permitted:** Standard real-world concomitant medications for symptom management and comorbidities. * **Prohibited:** Concurrent enrollment in an interventional clinical trial for RCC.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -14 to 0	Informed Consent, Medical History, Baseline IMDC Risk Score, EQ-5D questionnaire.
Baseline	Day 0	Treatment initiation of Nivolumab or Nivolumab + Ipilimumab.
Interim	Real-world clinical routine	Standard of care visits: Safety evaluation, investigator-assessed PFS/ORR per RECIST, HRQoL updates.
End of Study	Up to 5 Years	Final Overall Survival (OS) follow-up and safety data tracking completion.

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint * **Definition:** Overall Survival (OS) estimated over a 5-year follow-up period. * **Measurement Method:**

Tracking of real-world clinical records from the initiation of treatment until death from any cause.

7.2 Secondary & Exploratory Endpoints * Progression-Free Survival (PFS) (Investigator assessed or per RECIST). * Objective Response Rate (ORR). * Safety assessments and Health-Related Quality of Life (HRQoL) via the EQ-5D score.

8. Safety & Adverse Event Reporting

- **Adverse Event (AE) Monitoring:** Real-world safety data collection based on routine clinical patient reporting and EHR evaluations.
- **Serious Adverse Events (SAEs):** Captured through standard post-marketing pharmacovigilance pathways and real-world AE reporting criteria.
- **Data Monitoring Committee (DMC):** N/A (Observational, non-interventional study methodology).

9. Statistical Analysis Plan (SAP) Summary

- **Analysis Populations:** Intention-to-Treat (ITT) equivalent for real-world cohorts (all patients who started standard-of-care therapy).
- **Sample Size Justification:** Systematic collection of real-world effectiveness data recruiting 323 patients to provide a precise estimate for overall survival over a 5-year follow-up period.
- **Handling of Missing Data:** Kaplan-Meier methodology utilized to estimate OS and PFS, censoring subjects lost to follow-up or remaining event-free at data cutoff.

10. Quality Assurance & Regulatory Compliance

- **GCP Compliance:** This study will be conducted in accordance with Good Pharmacoepidemiology Practices (GPP) and local observational regulatory standards.
- **Monitoring Plan:** Real-world data/EHR abstraction verification per the prospective non-interventional design framework.
- **Audit Trail:** Electronic Case Report Forms (eCRF) and secure tracking of clinical pathway responses according to sponsor observational requirements.

11. Study Identifiers & Governance

- **Primary ID:** CA209-653
- **Registry Number:** NCT02940639
- **Sponsor/Lead Organization:** Bristol Myers Squibb
- **Study Title:** Real-Life Effectiveness and Safety in Participants With Advanced/Metastatic Renal Cell Carcinoma Starting 1st Line Nivolumab and Ipilimumab Combination Therapy or Nivolumab Monotherapy After Prior Therapy

- **Official Regulatory Title:** NORA Trial

12. Clinical Framework (Renal Cell Carcinoma Specific)

- **Primary Condition:** Advanced/Metastatic Renal Cell Carcinoma (aRCC)
- **Diagnosis Threshold:** Histologically or cytologically confirmed advanced/metastatic disease
- **Medical Methodology:** Standard of Care Oncology Treatment Pathway
- **Optimization Target:** Overall Survival (OS) and Tumor Objective Response

13. Study Procedures & Methodology

- **Management Pathway:** Standard clinical pathway for aRCC in Germany
- **Education Protocol (HCP):** Routine real-world clinical application of the approved product label
- **Education Protocol (Patient):** Standard oncology center patient counseling
- **Quality Audit Mechanism:** Investigator-assessed tumor reduction tracking and pharmacovigilance logging

14. Observation & Follow-Up Schedule

- **Total Duration:** 5-year overall survival follow-up
- **Onsite Visit Cadence:** As dictated by real-world standard of care treatment cycles
- **Remote Monitoring Frequency:** Routine data abstraction from existing clinical encounters
- **Checklist Review Interval:** Periodically tied to physician-determined clinical assessment milestones

15. Sign-off & Approval

Role	Name	Signature	Date
Principal Investigator	[Name]	_____	02-Apr-2026
Clinical Operations Lead	[Name]	_____	02-Apr-2026
Lead Statistician	[Name]	_____	02-Apr-2026